Vet. App. No. 19-0320

IN THE UNITED STATES COURT OF APPEALS
FOR VETERANS CLAIMS

DONALD R. FULTON,
Appellant,

v.

ROBERT L. WILKIE,
Secretary of Veterans Affairs,
Appellee.

ON APPEAL FROM THE BOARD OF VETERANS' APPEALS

BRIEF OF APPELLEE SECRETARY OF VETERANS AFFAIRS

MARK J. HAMEL
Appellate Attorney
Office of General Counsel (027K)
U.S. Department of Veterans Affairs
810 Vermont Avenue, N.W.
Washington, D.C. 20420
Attorney for Appellee
(202) 632-6135

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ISSUE PRESENTED

Whether the Court should affirm the November 8, 2018, decision of the Board of Veterans' Appeals (Board) that denied entitlement to service connection for cardiac disorder, to include coronary artery disease, atrial fibrillation, and peripheral vascular disease, claimed as ischemic heart disease.

STATEMENT OF THE CASE

A. Nature of the Case

Appellant, Donald Fulton, appeals the November 8, 2018, decision of the Board that denied entitlement to service connection for cardiac disorder, to include coronary artery disease, atrial fibrillation, and peripheral vascular disease, claimed as ischemic heart disease. Record Before the Agency (R.) at 9 (4-12).

The Board also granted entitlement to a separate, compensable rating for left foot ulcer. R. at 4. This favorable finding cannot be disturbed. See *Medrano v. Nicholson*, 21 Vet.App. 165, 170 (2007) ("The Court is not permitted to reverse findings of fact favorable to a claimant made by the Board pursuant to its statutory authority.").

The Board also remanded Appellant's claim of entitlement to a total disability rating based on individual unemployability due to service-

connected disability. R. at 4. The Court is without jurisdiction over this issue. See *Breeden v. Principi*, 17 Vet.App. 475, 478 (2004) (per curiam order) (stating that a Board remand decision is not a final decision over which this Court has jurisdiction); 38 U.S.C. §§ 7252(a), 7266(a).

B. Statement of Facts and Procedural History

Appellant served on active duty from August 1964 to August 1967, including service in the Republic of Vietnam. R. at 1292.

Appellant's July 1964 entrance medical examination indicated a sitting blood pressure reading of 110/70. R. at 2631 (2630-31). The examiner noted that Appellant's heart and vascular system were clinically normal. R. at 2630.

Appellant's June 1967 separation medical examination indicated a sitting blood pressure reading of 122/82. R. at 2629 (2628-29). The examiner noted that Appellant's heart and vascular system were clinically normal. R. at 2628.

In December 2004, thirty-seven years after his separation from service, Appellant underwent a cardiolite stress test at the Armstrong County Memorial Hospital. R. at 2332-33. The test report indicated that Appellant was admitted to the hospital with atrial fibrillation but "no prior history of cardiac problems." R. at 2332. The test results "did not show any changes suggestive of ischemia." R. at 2333.

Appellant's VA medical records indicate an onset of benign hypertension in 2004. R. at 2433 (2432-34).

SUMMARY OF THE ARGUMENT

The issue of entitlement to service connection for hypertension was not reasonably raised by the record because the medical evidence indicated that Appellant's heart and vascular system were clinically normal in service and Appellant's hypertension was first diagnosed thirty-seven years after his separation from service.

ARGUMENT

I. Standard of Review

"Whether an issue is reasonably raised by the record is essentially a question of fact, subject to the 'clearly erroneous' standard of review." *Lynch v. Wilkie*, 30 Vet.App. 296, 304 (2018) (citations omitted). "A finding is 'clearly erroneous' when although there is evidence to support it, the reviewing court on the entire evidence is left with the definite and firm conviction that a mistake has been committed." *Gilbert v. Derwinski*, 1 Vet.App. 49, 52 (1990) (citation omitted).

If the factfinder's account of the evidence is plausible in light of the record viewed in its entirety, the reviewing court may not reverse it even though convinced that had it been sitting as the trier of fact, it would have

weighed the evidence differently. *Id.* Where there are two permissible views of the evidence, the factfinder's choice between them cannot be clearly erroneous. *Id.*

II. The Board did not fail to address a reasonably raised theory.

The Board is required to consider all issues raised either by the claimant or by the evidence of record. *Robinson v. Peake*, 21 Vet.App. 545, 552 (2008) (citations omitted). However, this obligation "does not require the Board to assume the impossible task of inventing and rejecting every conceivable argument in order to produce a valid decision." *Id.* at 553. "[T]he Board is not required to conjure up issues that were not raised by the appellant." *Brannon v. West*, 12 Vet.App. 32, 35 (1998) (citation omitted); see also *Sondel v. Brown*, 6 Vet.App. 218, 220 (1994) (when issue is not reasonably raised, Board is not required to "conduct an exercise in prognostication").

Appellant argues that the Board failed to address a claim for hypertension that was reasonably raised by the evidence in Appellant's case. Appellant's argument is flawed for at least four reasons: (1) Appellant relies upon medical authorities that were not part of the record before the Board, (2) Appellant's theory is contrary to the medical evidence of record, (3) an isolated blood pressure reading is not clinically significant, and (4) the Board is not required to analyze all medical test results in the record

that might be elevated, but not abnormal, to conjure up issues that were not raised by a claimant.

First, Appellant improperly relies upon medical authorities that were not part of the record before the Board. The NAS report and the Merck Manual were not before the Board in November 2018. R. at 4-12. Therefore, this Court should decline to entertain Appellant's argument relying on these documents. See *Euzebio v. Wilkie*, No. 17-2879, 2019 U.S. App. Vet. Claims LEXIS 1476 (Aug. 22, 2019) (finding NAS reports were not constructively before the Board); 38 U.S.C. § 7252(b); *Rogozinski v. Derwinski*, 1 Vet.App. 19, 19 (1990) (holding that review in this Court shall be on the record of proceedings before the Secretary and the Board).

Second, Appellant's theory is contrary to the medical evidence of record. The Board noted that Appellant has a history of hypertension. R. at 9. However, the Board did not analyze a service connection claim for hypertension because there was no assertion or record evidence that Appellant's hypertension was related to his service. The pertinent medical evidence of record indicated that Appellant's hypertension is not related to his service: Appellant's July 1964 entrance medical examination and June 1967 separation medical examination each indicated that Appellant's heart and vascular system were clinically normal, and Appellant's VA medical records indicate an onset of benign hypertension in 2004, thirty-seven years after Appellant's separation from service. R. at 2433.

Third, an isolated blood pressure reading is not clinically significant. It is VA's policy to accept a hypertension diagnosis only on the basis of "readings taken two or more times [per day] on at least three different days." See *Gill v. Shinseki*, 26 Vet.App. 386, 390 (2013) ("Note (1) . . . states that hypertension must be confirmed by at least two blood pressure readings taken over several days"). This is consistent with the examiner's conclusion that Appellant's heart and vascular system were clinically normal in June 1967 and the medical conclusion that the onset of Appellant's hypertension was in 2004, thirty-seven years after his separation from service. R. at 2628.

Fourth, the Board is not required to analyze all medical test results in the record that might be elevated, but not abnormal, to conjure up issues that were not raised by a claimant. This view of the function of the Board is incorrect. Although it was common in the past for the Board to use its own medical judgment, this Court ended that practice in *Colvin v. Derwinski*, 1 Vet.App. 171, 172 (1991), and *Hime v. McDonald*, 28 Vet.App. 1, 5-6 (2016).

Given the evidence before the Board in this case, a claim of service connection for hypertension was not reasonably raised by the record.

CONCLUSION

For the foregoing reasons, the Secretary respectfully requests that the Court affirm the November 8, 2018, decision of the Board that denied entitlement to service connection for cardiac disorder, to include coronary artery disease, atrial fibrillation and peripheral vascular disease, claimed as ischemic heart disease.

Respectfully submitted,

s/ Mark J. Hamel
MARK J. HAMEL
Appellate Attorney
Office of General Counsel (027K)
U.S. Department of Veterans Affairs
810 Vermont Avenue, N.W.
Washington, D.C. 20420
Mark.Hamel@va.gov
Attorney for Appellee
(202) 632-6135

November 18, 2019

CERTIFICATE OF SERVICE

I certify that on November 18, 2019, I filed the foregoing Brief of Appellee Secretary of Veterans Affairs with the Clerk of the Court and served it on counsel for the other party by the Court's electronic filing system.

s/ Mark J. Hamel
MARK J. HAMEL